

Clearview: Destination Fertility

Lekki is the right destination. It is the wrong catchment. The growth is in the patients who cannot get this at home.

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How to read this document

This is not a plan and it is not a proposal. Nothing at Clearview has been counted yet, and I would rather be useful to you than confident in front of you. What this sets out is what I believe you are actually selling, why the marketing problem you described is a symptom rather than the disease, and the one position in this market that is both open and structurally yours.

Every number is a range, calibrated rather than measured. Section 11 lists what must be confirmed before any of it becomes a plan, and the discovery questionnaire that accompanies this document is how we confirm it.

If you read only three sections, read 4, 6 and 7.

1. The idea in one line

Nigeria exports patients for medical care. For this one specialty, Lekki should be importing them.

A travelling fertility patient needs two things that a fertility clinic cannot give her: a way to do this without living in Lagos for three weeks, and somewhere safe to be caught if it goes wrong. You own a laboratory and you own a hospital.

That is not a marketing message. It is a structural advantage, and none of the Lagos fertility clinics can copy it without buying a hospital.

2. Executive summary

What you told me. Nine years in, IVF is the cash cow, there is no steady marketing funnel for patient recruitment, and you want more stability and sustainability. You believe in systems and do not think you have one yet.

Where I disagree. You have a system and it has worked for nine years, which is longer than most fertility units in this country manage. The difficulty is that the system is you. Patients come on your name, referrals come from colleagues who know you personally, and the judgements live in your head rather than anywhere they can be taught or inherited. Stability and sustainability are another way of describing a business that still works in a week when you are not in it.

On marketing. In fertility the money is almost never lost between the advertisement and the enquiry. It is lost between the enquiry and the first cycle, between the first cycle and the second, and between the positive test and the delivery. Advertising before those three are closed buys more people into the same leak, so cost per cycle rises while margin falls.

The strategic error I think is being made. Clearview is being run as a Lekki clinic serving Lekki. Lekki is the most contested and most expensive fertility catchment in West Africa, and competing there on facility and advertising is the one fight where your advantages count for nothing. But Lekki is also the best **destination** in the region: the airport, the hotels, the safety, the concentration of specialists, and the familiarity. The catchment that matters is not the six kilometres around you. It is mainland Lagos, the rest of Nigeria, West Africa and the diaspora.

Why the hospital is the whole argument. Every fertility clinic in Lagos can treat a visitor. None of them can admit her. The specific fear of a travelling IVF patient is not the cycle failing, it is being unwell far from home with a doctor who does not know what was done to her. Owning the hospital converts that fear into



your strongest promise, and it is the reason this strategy is available to you and not to them.

What a win actually looks like. Four additional cycles a month from outside Lagos is in the region of **NGN 260 to 365 million a year**, on a laboratory you have already built and already paid for. That is the entire prize and it is a small number of families, not a mass market.

3. The reframe

3.1 What you believe the problem is

Clearview lacks marketing funnels, so recruitment is inconsistent, so revenue is lumpy, so the business feels unstable.

3.2 The reframe

Recruitment is the symptom. Three constraints bind, and all three sit inside a business you already own:

	The constraint	Why it binds
1	Conversion , enquiry to first cycle	Self pay, competing with a plot of land. The clinical decision takes one consultation; the financial decision takes months, involves family, and is shopped against three clinics. Most units convert around one enquiry in ten and cannot tell you the number.
2	Retention , one cycle to the next	Most cycles do not produce a live birth even in the best laboratories in the world. A unit built to sell one cycle hunts a stranger every month. A unit built to reach a baby over two or three attempts keeps the patient.
3	Catchment , who you are selling to at all	You are fishing in the most crowded pond in West Africa while holding the only boat that can reach the empty ones.

3.3 The asset we are positioning

Not a fertility clinic. A **destination** consisting of:

- An obstetrician and gynaecologist with nine years of independent IVF practice, who both achieves the pregnancy and is qualified to deliver it.
- An embryology laboratory already built, already paid for, and almost certainly carrying spare capacity. **Spare laboratory capacity is the cheapest growth in medicine**, because the marginal cost of one more cycle is consumables and an hour of skilled time, while the fixed cost is already sunk.
- **A hospital**, currently being run as a second business rather than as the thing that makes the first one safe to travel to.
- A frozen embryo store, which is a list of people who already trust you.

4. The four source markets

Sized by what a meaningful win costs rather than by market size, because Nigerian cycle-volume data is poor and a confident market figure here would be invented. The useful question is not how big the market is. It is how few families you need.

Annual revenue, NGN millions, from N additional cycles a month:

Extra cycles a month	Mainland Lagos	Rest of Nigeria	West Africa	Diaspora
1	57	66	71	91



Extra cycles a month	Mainland Lagos	Rest of Nigeria	West Africa	Diaspora
2	114	131	142	182
4	228	262	285	365
6	342	393	428	547

Travelling patients carry a higher package price than local ones, because the package includes accommodation, transfers, coordination and the safety net. Diaspora carries more again and is still a fraction of what they pay at home.

4.1 Mainland Lagos and the Lagos periphery

Why they come. They already travel across Lagos for specialist care and accept it. Lekki reads as the premium address.

What stops them. Traffic, and the monitoring burden. A stimulation cycle needs near daily scans and bloods for roughly two weeks, and no woman with a job is crossing the Third Mainland Bridge fourteen times.

What has to be true. A monitoring arrangement that is not in Lekki. Either your own scanning points, or partner gynaecologists on the mainland who do the monitoring while you do the retrieval and transfer.

4.2 The rest of Nigeria

Why they come. Most states have no IVF unit or one low volume one. Abuja and Port Harcourt have capacity, much of the rest of the country effectively does not, and patients already fly to Lagos for other specialties.

What stops them. The same monitoring problem, but worse, because the standard answer is to relocate to Lagos for three weeks. That is unaffordable, unexplainable to an employer, and for many women unexplainable to a family she has not told.

What has to be true. A shared care model, which is the single most important operational idea in this document. The patient does her workup and her monitoring scans with a partner gynaecologist in her own city, and flies to Lekki for four or five days covering retrieval and transfer. **That reduces the trip from three weeks to under a week and it is what makes domestic fertility travel work anywhere in the world.** Almost nobody in Nigeria runs it properly.

What kills it. Paying partner doctors for referrals rather than for clinical work. That is a professional conduct problem long before it is a commercial one, and it must be structured as paid monitoring work under a written protocol.

4.3 West Africa

Why they come. Lagos is the regional medical hub. Francophone West Africa in particular has very limited assisted reproduction capacity, ECOWAS movement removes the visa barrier for much of the region, and the alternative for those who can afford it is Europe or North Africa at several times the cost.

What stops them. Language, payment and FX, and having no reason to believe a Lagos clinic over a Paris one.

What has to be true. French language capability in the coordination team, a payment route that works across borders, published outcome data, and a small number of named partner gynaecologists in Accra, Abidjan, Lomé, Cotonou and Douala. This is a relationship business, built one doctor at a time, and it is slower than the others.

4.4 The diaspora



The most valuable and the most misunderstood of the four. **The reason to come is different in each country, and marketing to them as one group is the standard mistake.**

	Why they come	What it turns on
United States	Overwhelmingly price. A cycle is NGN 22 to 38 million equivalent privately, against roughly NGN 7.6 million for a full package here. The saving is NGN 14 to 30 million per cycle, which pays for the whole trip several times over.	Do they believe the clinic. Price is already settled.
United Kingdom	Not mainly price. The saving is real but thinner, somewhere between NGN 1 and 8 million. What actually drives it is rationing of funded cycles, age limits, and how long the waiting is.	Access and time, not cost.
Both	Wanting a Nigerian doctor, wanting to combine treatment with a visit home, and donor availability . Egg donors of Black African ancestry are severely scarce in both countries, with waits measured in years. Nigeria has an available donor pool.	This is structural and very hard for a Western clinic to copy.

What stops them. Trust, entirely. They will ask for outcome data, for accreditation, for the name of the embryologist, and for what happens if something goes wrong five thousand miles from their own doctor. **Every one of those questions is answered better by a group that owns a hospital than by a clinic that does not.**

The donor point needs handling carefully. It is a genuine and defensible advantage and it is also the area of this field with the most serious ethical exposure, particularly around donor recruitment, compensation and consent. If Clearview builds here it must be visibly the most careful operator in the market, not the most aggressive one, because the first scandal in Nigerian donor gametes will take down whoever is nearest to it.

5. What a travelling fertility patient actually needs

This is the operational heart of the strategy, and it is where a clinic wins or loses the traveller.

Her problem	What most clinics offer	What a destination offers
She cannot be in Lagos for three weeks	Relocate, or do not come	Shared care: monitoring at home, four to five days here
She is coming to a city she may not know	A clinic address	Accommodation, airport transfer, a named person who meets her
She is terrified of being unwell far from home	Good luck	Admission to your own hospital, by your own team, included in the package
She has one shot at the trip	A cycle	A plan that includes what happens if this cycle fails, agreed before she flies
She conceives, then flies home	Discharge at eight weeks to a stranger	Continuity, shared antenatal protocol with her own doctor, and the option to deliver here
She wants another child in three years	Nothing	Her embryos are already in your tank

The complication that decides everything



Ovarian hyperstimulation syndrome typically declares itself several days **after** retrieval, which is precisely when a travelling patient has flown home. A woman who becomes unwell in Kano or Accra, under a doctor who does not know what drugs she was given or how many eggs were collected, is the single scenario that keeps fertility travel small in this region.

A group that owns a hospital can say something no Lagos fertility clinic can say: **if anything goes wrong, you are admitted here, by us, and it is already in the price.** That one sentence is worth more than any campaign, and it is true today.

6. The competitive map: two flags planted, an open centre

Flag one, the national institutions. The long established names, among them Nordica, The Bridge Clinic and Medical ART Center, own age, scale, multiple sites and national recognition. A patient wanting the safest known brand goes to them. You will not take that position and should not spend a naira trying.

Flag two, the Lekki and Victoria Island boutiques. Newer, well finished, competing on facility, privacy and concierge experience. Well funded. This is the fight you are in today by default and it is the most expensive one available, because it is won with fit out and advertising.

The open centre. Every operator above is a standalone fertility clinic serving people who live nearby. Their model ends at the pregnancy scan and begins at their front door. Nobody is built for a woman who does not live here, and nobody can admit her if she becomes unwell.

A clinic cannot copy this without buying a hospital and recruiting obstetricians. A hospital cannot copy it without building an embryology laboratory and finding an embryologist, which takes years. You are an obstetrician who already owns both. That is a moat, and moats are rare in this sector.

7. Recommended positioning

7.1 The spearhead: come to us, and we will not hand you over

Clearview is where a woman who does not live in Lekki can have IVF without moving her life to Lagos, with one named team from the first consultation to six weeks after the birth, and a hospital of our own behind her the entire time.

Everything ladders to that. It is true today, it is verifiable, and it is the one thing your competitors cannot say.

7.2 The voice: honest about the odds

The reputational wound in Nigerian fertility is the suspicion that clinics sell cycles to people who cannot benefit. The most powerful voice available to you is the opposite of a sales voice: telling a woman she needs a myomectomy before a cycle, or that at her age and hormone profile the honest number is this and not that, or that she should not do a third cycle with her own eggs.

An obstetrician and gynaecologist can say those things with an authority a fertility clinic cannot. Evidence from comparable markets is consistent that patients rank manner and communication above technical quality and well above price. Honesty is not a softening of the commercial position. It is the commercial position, and for a patient deciding whether to get on a plane it is the only thing she has to go on.

7.3 The reassurance: the whole journey, in one place

Not the lead, but the thing that settles it. Investigation, treatment, pregnancy, delivery, the newborn, and the sibling from the same frozen embryos in three years.

8. Brand architecture: the opposite of the usual answer



For a surgeon, the person is the product and the institution supports it. **For Clearview I would invert that deliberately.**

Brand	Role	Why
Clearview (the group)	Primary. The hero brand carrying the destination and continuity promise across both businesses.	The brief is that the business must work without you. Building your personal name deeper makes the problem you asked me to solve worse. A patient in Abidjan or Houston is choosing an institution, not a man.
Clearview Fertility	Supporting: the specialist capability	Keeps clinical credibility distinct, and lets the laboratory serve more than one front door.
Clearview Hospital	Supporting: the safety net and the delivery	Today it reads as a separate business. It is the reason the whole strategy works.
Dr Kunle Ajayi	Secondary. The named clinical authority and trust anchor, always presented at Clearview.	Your reputation is real and should be used. It should not be the thing the business rests on.

Two further practical reasons:

- **A destination promise cannot be carried by one man's name**, because the promise is precisely that it does not depend on one man being available on the day someone lands.
- **Your surname is the most crowded one in Nigerian fertility.** Several of the best known IVF founders in Lagos share it, so a patient searching your name meets your competitors. That is a discoverability liability rather than an asset. *(To be confirmed before it is relied on anywhere public.)*

9. The five moves that make the competition irrelevant

Move one. Publish your real outcomes, by age band. Every clinic in Lagos claims high success and none publishes anything a patient can interrogate. For a woman deciding whether to board a plane, this is not a nice-to-have, it is the whole decision. Being first to publish honest age banded live birth figures is the strongest trust instrument in this market and it is free. **It is also the most dangerous move if the numbers are not good, so it must be evidenced before it is claimed.** Measure this first.

Move two. Build the shared care network. Named partner gynaecologists in Ikeja, Abuja, Port Harcourt, Benin, Enugu, then Accra and Abidjan, doing workup and monitoring locally under a written protocol, paid properly for clinical work. This is what turns a three week relocation into a five day trip, and it is the operational unlock for three of your four source markets.

Move three. The safety net, written into the package. Admission to Clearview Hospital for any complication of a cycle we performed, included. Nobody else in Lagos can offer it and it directly answers the one fear that stops people travelling.

Move four. Price the journey, not the cycle. Fixed price packages covering investigation through to delivery, with instalment payment, and separate packages for the traveller that include accommodation and transfers. The family is buying a baby, not an attempt. **Requires move one first**, because a package covering the patients who need three attempts cannot be priced without knowing how many will.

Move five. The sibling programme. Every woman who has a baby with you and has embryos in your tank is a second child waiting two or three years. Nobody treats that as a programme. It is the longest tail and highest margin revenue available and it is a relationship, not a campaign.

10. The order I would do it in



You asked for funnels. Funnels are the fifth thing, not the first.

	Do this	Because
1	Measure outcomes and count the funnel	Everything else is unpriceable and unclaimable until this exists
2	Close the conversion gap and capture the downstream	It buys cycles and obstetric revenue from demand you already generate
3	Build the recall and the tank	Best margin in the building, no new patient needed
4	Stand up shared care, one partner city at a time	This is the growth engine
5	Then market, against a position worth marketing	By then every naira lands in a business that holds what it catches

11. What has to be confirmed before any of this is a plan

These are not things I could look up. They decide whether the position above is available.

What we need	How it gets answered	What it decides
Live birth rate by age band, from your records	Retrospective pull over two to three years	Whether the destination strategy is available at all. A traveller buys outcomes.
Enquiries a month, and what happens to each	Count for eight weeks against one sheet	Demand problem or conversion problem. Opposite answers.
Where your patients already travel from	Ask every patient at booking for eight weeks	Whether inbound is a new idea or a description of what is already happening quietly
True laboratory capacity and how deep the team is	How many retrievals a week is the ceiling, and how many people can run a day independently	Caps every growth option, and it is what an investor examines first
Where your IVF pregnancies deliver	Cross match the pregnancy list to the delivery register	The size of the downstream number
Patients with embryos in storage, and who has been called	A day in the laboratory and storage records	The fastest revenue here, and the consent exposure behind it
Whether the two companies are one business or two	Accounts, shareholdings, how money moves between them	Whether downstream capture is a real gain or a transfer between your own pockets
Drug and consumable cost as a share of cycle price	Twelve months of invoices against twelve months of billing	How exposed the margin is to the currency, which matters more when pricing in foreign markets
What the Lekki competitors and the Accra and London options actually charge and promise	A morning of price shopping, done properly	You cannot position against an offer you have not read



What we need	How it gets answered	What it decides
Your current regulatory and accreditation position	Registration, protocols, what you could show an international patient	International patients ask. Nigerian ART regulation is thin, so a destination clinic must visibly self regulate.

The accompanying discovery questionnaire covers all of it. Most is two to three weeks of work and none of it commits you to anything. At the end you have either a strategy built on measured facts or an early and inexpensive reason not to proceed, and both are worth having.

12. The first conversation

The natural first step is a conversation about sections 4, 6 and 8, because those are the three places where I may be telling you something you disagree with, and that is much better discovered over coffee than in month fourteen.

Bring your cycle register and your last twelve months. If the leaks are where I believe they are, you will have found the money before you have spent any.

Consult for Africa builds and runs health businesses in Nigeria. We have taken facilities through licensing, structured medical joint ventures and shareholder agreements, rebuilt tariff and payor strategies, run organisational audits for founder led practices at roughly the stage Clearview is at now, and recruited consultants, including from the diaspora.